

Adrabetadex Provider Net Cost Recovery Model

Concept Review Brief

Adrabetadex needs a provider net cost recovery model because gross WAC does not show whether a treatment center is financially whole after acquisition cost, reimbursement, operating burden, claim friction, and payment timing.

WAC / ADMIN \$39,000	ANNUAL ADMIN 26	ANNUAL GROSS WAC \$1.014M	340B PROXY \$29,991	INITIAL PART B REF. \$40,170
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Why the model is beneficial

Launch question	What the calculator makes visible	Benefit
Can a provider afford to start treatment?	Drug acquisition, reimbursement, and timing exposure by scenario	Identifies where uptake is workable, thin, or blocked
Does WAC overstate provider economics?	WAC, acquisition basis, reimbursement, and net recovery as separate layers	Prevents list price from being treated as provider margin
Which sites should be prioritized?	COE/HOPD, non-340B, office, Medicaid, and white-bagging pathways	Supports practical launch sequencing
What support changes the result?	Support required to move a negative case to neutral	Ties support design to a specific provider gap

Provider economics bridge

Layer	What changes	Model output
Gross exposure	WAC multiplied by dosing cadence	Per-dose and annual exposure
Acquisition basis	WAC, 340B proxy, support, or no provider acquisition	Provider cost at risk
Reimbursement basis	Payer/site benchmark, contract, state method, or interim-code logic	Expected allowed payment
Operating burden	Labor, handling, billing/admin, financing, denial rework	Margin erosion
Net recovery	Reimbursement plus non-drug contribution minus total burden	Workable, thin, negative, or state-dependent

Scenario signal

Scenario	Acquisition	Reimbursement basis	Draft signal
340B COE / HOPD	\$29,991	\$40,170 benchmark logic	Workable
Non-340B HOPD	\$39,000	\$40,170 benchmark logic	Thin to negative
Commercial buy-and-bill + 5% support	\$37,050	Medical benchmark	Potentially neutral
Commercial buy-and-bill + 10% support	\$35,100	Medical benchmark	More consistently neutral
Commercial white-bagging	\$0 provider acquisition	Administration only	No drug spread
Medicaid non-340B	State-specific	PAD or MCO logic	High variance

Comparator use and review boundaries

Comparator products are used to test reimbursement mechanics under the same scenario. They are not used to imply clinical equivalence.

Spinraza comparator as intrathecal analogue

Spinraza point	Public evidence	Why it is used
Intrathecal route	Confirmed in U.S. label [S1]	Shared administration context
Billing unit	J2326, 0.1 mg unit [S2]	Drug-to-dose translation discipline
Procedure sequence	CPT 96450 commonly billed same encounter [S3]	Drug and procedure economics are separate
WAC/list context	Recent public list-price references reported by Reuters [S4]	Market reference only
ASP reporting status	ASP file status does not determine coverage [S6]	Explicit data-availability constraint

Adrabetadex vs intrathecal analogue framing

Economic layer	Adrabetadex	Spinraza analogue	What this demonstrates
Gross price input	\$39,000 per administration	Public WAC/list references available [S4]	Gross price is only the first input
Billing structure	Product-specific code pending	J2326, 0.1 mg unit [S2]	Coding structure affects claim translation
Payment benchmark	Initial WAC-based reference, payer-specific thereafter	ASP/payment visibility may vary [S6]	Payment logic must be modeled separately
Acquisition basis	WAC, 340B proxy, support, or channel path	Site/channel dependent	Provider cost is not automatically WAC
Net result	Scenario-calculated	Scenario-calculated if inputs are available	Provider economics are modeled, not assumed

Review boundary

Appropriate statement	Boundary
The model compares acquisition, reimbursement, operating burden, and payment timing	It does not guarantee coverage or payment
The model tests 340B, non-340B, Medicaid, commercial, and white-bagging scenarios	It does not predict payer behavior
The model estimates support needed to close a provider shortfall	It does not recommend final price
Spinraza is used as an intrathecal reimbursement analogue	It does not imply clinical equivalence

Source notes

ID	Source
S1	Spinraza U.S. prescribing information
S2	CMS HCPCS reference for J2326, nusinersen, 0.1 mg
S3	CMS/Noridian billing guidance for Spinraza
S4	Reuters, March 30, 2026 reporting on Spinraza public list-price references
S6	CMS ASP pricing-file policy that ASP file presence or absence does not prove coverage status
S8	42 CFR 414.904(e)(4), initial-period cap when ASP is unavailable